

50 providers · 5 months on Abridge · 70% utilization · 42,000 documented encounters

AT A GLANCE

Your 50 providers are generating \$4K each annually in documented value. At 70% utilization, you're capturing \$208K today—with a clear path to \$738K at scale.

\$208K

ANNUAL VALUE CREATED

Defensible hard dollars

\$4K

VALUE / PROVIDER

Based on 50 providers

+\$530K

EXPANSION POTENTIAL

At 150 providers

DEPLOYMENT MATURITY

Utilization Rate

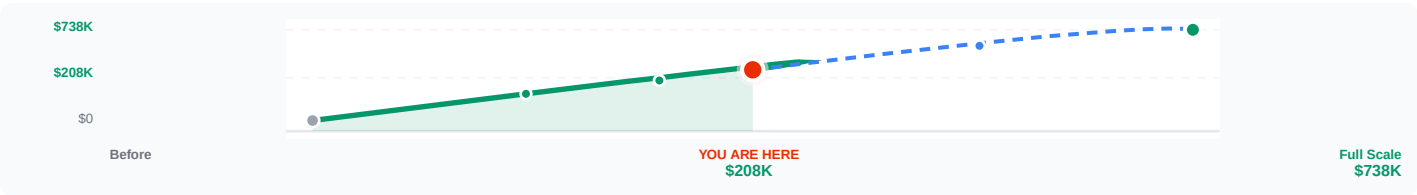
70%

Developing

VALUE DRIVERS

wRVU Per Encounter

YOUR VALUE JOURNEY



VALUE BREAKDOWN

Core Financial Value Directly measurable revenue impact	\$208K
wRVU Per Encounter 2,000 encounters × \$33/wRVU × 50% attribution	
	\$208K

KEY TAKEAWAY

The foundation is solid. Your deployment is proving out the value model in your environment—the next step is deepening adoption among existing providers and extending to new ones.

● **WRVU PER ENCOUNTER**
Revenue capture improvement

\$208K
annual value

BEFORE ABRIDGE

2.10
wRVU/enc

→

AFTER ABRIDGE

2.40
wRVU/enc

=

+0.30
14% lift

BENCHMARK COMPARISON
Typical range: 3-7% lift

● Above typical range

VALIDATION RECOMMENDED
Your results exceed our typical range. This isn't necessarily wrong—some organizations do see higher lift, especially if baseline documentation was sparse. But we recommend confirming: Was baseline measured the same way as current? Were there other coding or documentation initiatives during this period? Is the provider population consistent between periods?

VALUE CALCULATION
+0.30 wRVU/enc × 42,000 encounters × \$33/wRVU × 50% attribution = \$208K
We use Medicare's \$33 conversion factor and 50% attribution to Abridge. If your payer mix is commercial-heavy (where conversion factors run \$45-65), actual revenue impact may be 30-50% higher.

WHAT THIS MEANS
Here's what we're seeing: A 14.3% lift in wRVU typically means your documentation is now capturing clinical complexity that was always there—providers aren't doing more, the notes are just reflecting reality better. Your 14% lift is above our typical 3-7% range, which is interesting. This could mean several things: your baseline documentation may have been particularly sparse, or you have complex patient populations that benefit more from thorough capture. It's worth exploring—if you can validate the baseline methodology, this becomes a powerful proof point.

🔍 EARLY INDICATORS

✔️ WHAT'S WORKING WELL

- Revenue capture is responding—14% wRVU lift suggests documentation is better reflecting clinical complexity

🔍 AREAS TO WATCH

- Utilization at 70%—worth exploring what's keeping remaining providers from adopting. Training gaps? Specialty-specific barriers?
- wRVU lift above typical—exciting if validated. Consider auditing baseline methodology to confirm this is real lift vs. measurement change
- Time savings currently unrealized—the hours are there, but haven't yet converted to patient access or reduced overtime. Worth a conversation with ops.

📈 OPTIMIZATION OPPORTUNITIES

1 DEEPEN ADOPTION

+\$31K/yr

CURRENT

70% of providers using consistently



TARGET

80-85% (where value compounds)

RECOMMENDED ACTION

Look for patterns: Which specialties or sites are lagging? Often there are 2-3 common barriers that, once addressed, unlock the next wave of adoption.

2 VALIDATE YOUR STANDOUT RESULTS

+\$0/yr

CURRENT

14% wRVU lift (above typical)



TARGET

Confirmed, defensible methodology

RECOMMENDED ACTION

High-lift results are great news if they hold up to scrutiny. Quick audit: Was baseline measured consistently? Any other coding initiatives running in parallel? If you can answer "yes" to both, this becomes a compelling proof point.

📊 THE BIGGER PICTURE

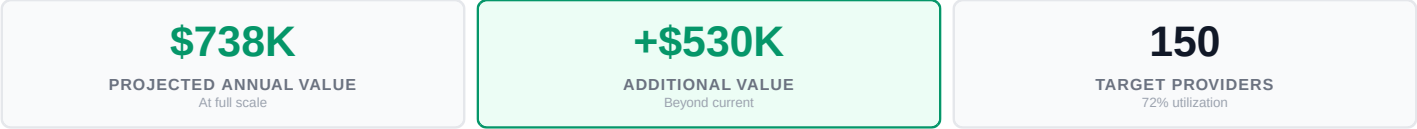
At 70% utilization, you've moved past the early adoption phase into optimization. The focus shifts from "does this work?" to "how do we maximize it?"—protecting these gains while extending to new areas.

KEY TAKEAWAY We've identified 2 optimization areas worth exploring. Start with whichever feels most actionable—even partial progress on any of these moves the needle meaningfully.

THE PATH TO FULL SCALE

THE EXPANSION STORY

You've established proof of concept. The question now is: how does this scale? The math suggests each additional provider at target utilization adds roughly \$5K annually.



SCALING MATH

Current annual value	\$208K
× Provider scale (-50, 150-)	3.0x
× Utilization improvement (70% '72%)	1.03x
Projected annual value at scale	\$738K

CURRENT VS. PROJECTED



Think of your current deployment as a pilot that's already delivering. The patterns you're seeing at 50 providers translate predictably as you scale—this isn't a leap of faith, it's an extension of demonstrated results.

KEY TAKEAWAY The model works in your environment. The expansion opportunity is about replicating what's already proven—methodically extending to new providers while deepening adoption with existing ones.

HOW WE CALCULATED YOUR VALUE

Every calculation here traces back to either your data or industry-standard benchmarks. We lean conservative throughout—no optimistic assumptions. The methodology is transparent so you can validate the logic yourself.

YOUR INPUTS

- 50 providers
- 60,000 annual encounters
- 70% utilization rate
- 5 months on Abridge
- 42,000 documented encounters

VALUE ATTRIBUTION

- wRVU: \$33/wRVU (Medicare), 50% attribution
- Time: \$150/hr provider value
- Retention: \$500K per departure
- Commercial payers: 30-50% higher

BENCHMARK RANGES

TYPICAL ABRIDGE RESULTS

- wRVU lift: 3-7%
- Time reduction: 3-5 min/enc
- Chart closure: +5-15 pp

QUALITY OF LIFE

- Pajama time: -2-5 hrs/wk
- Satisfaction: +10-20 pts
- Burnout: varies by org

WHAT WE DON'T INCLUDE

- Downstream revenue from patient experience
- Quality measure incentives (MIPS, HEDIS)
- Reduced compliance/audit risk
- Training time reduction & career satisfaction

Have questions about any calculation? We're happy to walk through the details—every number here is designed to be defensible in a CFO conversation.

KEY TAKEAWAY Conservative by design: with commercial payer rates and full attribution, actual value is typically 30-50% higher than shown here.